

Guidelines on Antibiotic Prophylaxis for the Prevention of Infective Endocarditis in Dental Procedures

A synthesis of the AHA 2007 Guideline (updated by the 2021 AHA Scientific Statement) as adapted for dental practice, endorsed for clinical use by the ADA Council on Scientific Affairs.

Test Corpus Document — For Ingestion Pipeline Evaluation Only, Not for Clinical Use

1. Purpose and Scope

This document summarizes recommendations for antibiotic prophylaxis against infective endocarditis (IE) in patients undergoing dental procedures that involve manipulation of gingival tissue, the periapical region of teeth, or perforation of the oral mucosa. Antibiotic prophylaxis is recommended only for the subset of cardiac patients at highest risk of an adverse outcome from IE; it is no longer recommended for patients whose only risk factor is an increased lifetime risk of acquiring IE.

2. Cardiac Conditions for Which Prophylaxis IS Recommended

- Prosthetic cardiac valve or prosthetic material used for cardiac valve repair
- Previous infective endocarditis
- Unrepaired cyanotic congenital heart disease (CHD), including palliative shunts and conduits
- Completely repaired congenital heart defect with prosthetic material or device, during the first 6 months after the procedure
- Repaired CHD with residual defects at or adjacent to the site of a prosthetic patch or prosthetic device
- Cardiac transplant recipients who develop cardiac valvulopathy

3. Dental Procedures Requiring Prophylaxis

All dental procedures that involve manipulation of gingival tissue or the periapical region of teeth, or perforation of the oral mucosa. Prophylaxis is NOT recommended for routine anesthetic injections through non-infected tissue, dental radiographs, placement/adjustment of removable prosthodontic or orthodontic appliances, adjustment of orthodontic brackets, shedding of deciduous teeth, or bleeding from trauma to the lips or oral mucosa.

4. Recommended Prophylactic Regimens — Single Dose 30–60 Minutes Before Procedure

Situation	Agent	Adult Dose	Pediatric Dose	Route
Standard general prophylaxis	Amoxicillin	2 g	50 mg/kg (max 2 g)	Oral, 30-60 min prior
Unable to take oral medication	Ampicillin	2 g	50 mg/kg (max 2 g)	IM or IV, within 30-60 min prior
Unable to take oral medication	Cefazolin or Ceftriaxone	1 g	50 mg/kg (max 1 g)	IM or IV, within 30-60 min prior
Allergic to penicillins/ampicillin (oral)	Cephalexin*	2 g	50 mg/kg (max 2 g)	Oral, 30-60 min prior
Allergic to penicillins/ampicillin (oral)	Azithromycin or Clarithromycin	500 mg	15 mg/kg (max 500 mg)	Oral, 30-60 min prior

Allergic to penicillins/ampicillin (oral) — alt.	Doxycycline	100 mg	≥ 45 kg: 100 mg; < 45 kg: 2.2 mg/kg	Oral, 30-60 min prior
Allergic to penicillins/ampicillin and unable to take oral medication	Cefazolin or Ceftriaxone	1 g	50 mg/kg (max 1 g)	IM or IV, within 30-60 min prior

*Cephalosporins should not be used in an individual with a history of anaphylaxis, angioedema, or urticaria with penicillins or ampicillin. No data demonstrate superiority of any one oral cephalosporin over another for endocarditis prophylaxis; generic cephalexin is widely available and inexpensive.

5. Discontinuation of Clindamycin as a Recommended Alternative

The 2021 AHA Scientific Statement discontinued the recommendation of clindamycin as an oral or parenteral alternative regimen for patients allergic to penicillins or ampicillin. This change reflects data suggesting clindamycin may cause more frequent and severe adverse reactions—including a higher risk of *Clostridioides difficile* infection—than the alternative agents listed in Section 4. Clinicians who identify a patient chart still listing clindamycin 600 mg as the penicillin-allergy alternative should update the regimen to azithromycin, clarithromycin, cephalexin, or doxycycline as appropriate.

6. Missed Dose Prior to Procedure

If the dosage of antibiotic is inadvertently not administered before the procedure, it may be administered up to 2 hours after the procedure. Administration of prophylaxis after this window provides negligible benefit and is not recommended.

APPENDIX A — SCANNED ADDENDUM (LEGACY 2008 CHART, OCR TRANSCRIPTION)

The following text reproduces a legacy clinic wall-chart as transcribed by OCR from a photocopy. Formatting, spacing, and line breaks are preserved as-is, including scan artifacts, to test parser robustness against messy real-world source documents.

AMER. HEART ASSOC / A.D.A. -- QUICK REF CARD (rev. 07/2008) -- PHOTOCOPY, POOR QUALITY

STANDARD RX.....AMOXICILLIN....2GM PO....1HR PRE-OP

PCN ALLERGY(PO).....CLINDAMYCIN....600mg PO....1HR PRE-OP [SEE NOTE 1]

PCN ALLERGY(PO).....CEPHALEXIN.....2 G.....1 HR PRE-OP

PCN ALLERGY(PO).....AZITHROMYCIN...500mg.....1hr pre-op

NPO PT.....AMPICILLIN.....2gm IM/IV...W/I 30 MIN

NPO+PCN ALLERGY.....CLINDAMYCIN....600mg IV...W/I 30 MIN [SEE NOTE 1]

PEDS DOSING (<45kg, calc by wt, DO NOT EXCEED ADULT DOSE):

AMOXICILLIN..... 50mg / kg

CEPHALEXIN..... 50mg / kg

CLINDAMYCIN..... 20mg / kg [SEE NOTE 1]

AZITHROMYCIN..... 15mg/ kg

NOTE 1 -- SUPERSEDED. See Section 5 of main guideline (2021 update): clindamycin

is NO LONGER a recommended alternative regimen. This wall-chart is

retained in the archive for historical / OCR-testing purposes only

and does NOT reflect current prophylaxis recommendations.

card printed by: Dept. of Oral & Maxillofacial Surgery, rm 4B QTY 4 remaining

References: Wilson W, Taubert KA, Gewitz M, et al. Prevention of Infective Endocarditis: Guidelines from the American Heart Association. Circulation. 2007;116:1736-1754. — Wilson WR, Gewitz M, Lockhart PB, et al. Prevention of Viridans Group Streptococcal Infective Endocarditis: A Scientific Statement From the American Heart Association. Circulation. 2021;143:e963-e978. — American Dental Association, Oral Health Topics: Antibiotic Prophylaxis Prior to Dental Procedures.